

PRIOR AUTHORIZATION APPEAL

Formal Medical Appeal — Oncology

TO:	UnitedHealthcare — UnitedHealthcare Medical Director — Prior Authorization Appeals
DATE:	March 26, 2026
RE: Drug:	Enhertu (trastuzumab deruxtecan)
Denial Type:	NOT MEDICALLY NECESSARY
Reference #:	PA-2026-4491827
Patient ID:	PT-2026-001
Member ID:	UHC-ENC-XXXX

ATTACHMENTS ENCLOSED

1. NCCN Breast Cancer v2.2026 — BRE-M p.10
2. FDA Package Insert — Enhertu (NDA 761139)
3. Prior Therapy Summary
4. Original Denial Notice dated 2026-03-10
5. Patient Biomarker Report

I. EXECUTIVE SUMMARY

This appeal requests immediate reversal of UnitedHealthcare's denial of Enhertu (trastuzumab deruxtecan) for a patient with Stage IV breast cancer. The denial, citing "not medically necessary", directly contradicts Breast Cancer v2.2026 Category 1 recommendation and FDA approval for this indication.

The requested therapy is FDA-approved specifically for this patient population. The conflict has been identified as Type B: FDA approved Aug 2022 specifically for HER2-low. NCCN Category 1 recommendation. UHC policy only recognizes HER2-positive (3+ or ISH+), directly contradicting the FDA indication and NCCN guideline for HER2-low..

This denial constitutes bad-faith delay of life-saving oncology treatment and may violate New York Insurance Law § 2601 (unfair settlement practices) and NY Public Health Law § 4903 (utilization review requiring medically necessary care approval within 72 hours for urgent requests).

II. CLINICAL BACKGROUND

The patient presents with Stage IV breast cancer with the following biomarker profile: {"HER2": "low (IHC 1+)", "HR": "positive", "ESR1": "wildtype"}. Prior therapies include: [{"drug": "anastrozole", "start": "2024-01", "end": "2024-08", "response": "progression"}, {"drug": "fulvestrant", "start": "2024-09", "end": "2025-01", "response": "progression"}]. Current performance status: ECOG 1.

Given the documented disease progression and biomarker profile, Enhertu (trastuzumab deruxtecan) represents the evidence-based, guideline-concordant next line of therapy. No alternative therapies in the same class are available that match this patient's molecular profile.

Delaying access to this therapy risks disease progression, metastatic spread, and irreversible harm to this patient's survival outcomes.

III. NCCN & FDA GROUND TRUTH — CLINICAL STANDARDS

The Breast Cancer v2.2026 — the authoritative oncology treatment standard — designates trastuzumab deruxtecan as a Category 1 recommendation (preferred) for this exact indication: Metastatic HER2-low breast cancer in patients with biomarker profile {"HER2":"low","HR":"positive"}.

NCCN Category 1: Trastuzumab deruxtecan is the preferred regimen for HER2-low metastatic breast cancer in the second-line setting. Category 1 reflects uniform consensus based on high-level evidence.

FDA approval was granted for FDA approved for unresectable or metastatic HER2-positive breast cancer (Aug 2019) and HER2-low (Aug 2022) after prior chemotherapy. (Reference: NDA 761139). This approval constitutes the highest regulatory standard of evidence for efficacy and safety in this population.

NCC Category 1 indicates uniform consensus based on high-level evidence — there is no credible clinical or scientific basis for the payer's denial classification.

%¶ **NCCN CITATION**

NCCN Breast Cancer v2.2026 — BRE-M p.10

IV. PAYER POLICY CONTRADICTION ANALYSIS

UnitedHealthcare policy MP-ONC-1234 states: "Not covered for HER2-low (IHC 1+, IHC 2+/ISH-) as clinical benefit not established per plan criteria". This language directly contradicts:

1. FDA-approved indication for Enhertu covering this exact patient population.
2. Breast Cancer v2.2026 Category 1 recommendation.
3. The peer-reviewed clinical evidence base underlying both of the above.

Conflict type B: FDA approved Aug 2022 specifically for HER2-low. NCCN Category 1 recommendation. UHC policy only recognizes HER2-positive (3+ or ISH+), directly contradicting the FDA indication and NCCN guideline for HER2-low.

This denial appears to be a systematic policy that creates coverage barriers for guideline-concordant oncology care — a pattern that has been identified across multiple plans and geographies. Such systematic barriers may constitute bad-faith insurance practices.

V. LEGAL FRAMEWORK & BAD-FAITH ANALYSIS

Under ERISA Section 502(a)(1)(B) (29 U.S.C. § 1132), the patient has the right to recover benefits wrongfully denied under the plan. The plan's denial, which contradicts NCCN Category 1 guidelines and FDA regulatory standards, exceeds the plan's lawful authority to define medical necessity contrary to established clinical evidence.

Applicable state law: New York Insurance Law § 2601 (unfair settlement practices) and NY Public Health Law § 4903 (utilization review requiring medically necessary care approval within 72 hours for urgent requests)

"This denial contradicts Breast Cancer v2.2026, BRE-M p.10. Continued denial constitutes bad-faith delay of life-saving oncology treatment and may constitute a violation of New York Insurance Law § 2601 (unfair settlement practices) and NY Public Health Law § 4903 (utilization review requiring medically necessary care approval within 72 hours for urgent requests)."

NOTICE TO MEDICAL DIRECTOR:

Continued denial of this FDA-approved, NCCN Category 1 therapy constitutes bad-faith delay of life-saving oncology treatment. This notice serves as formal documentation of bad-faith practices under applicable state and federal law.

VI. REQUESTED RESOLUTION

We respectfully demand immediate approval of Enhertu (trastuzumab deruxtecan) for the above-referenced patient and reversal of denial reference PA-2026-4491827. This request should be treated as urgent given the oncologic context.

Per ERISA regulations and applicable state law, a decision must be rendered within 72 hours for urgent/expedited appeals or 30 days for standard appeals. Failure to respond within this timeframe will be treated as a deemed denial and will be escalated to the appropriate state insurance regulatory authority and, if necessary, federal courts.

Respectfully submitted,

Authorized Clinical Representative

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